> tance of the therapist-patient relationship for patient change. The better the relationship: (1) the more open is the patient about his feelings, (2) the more likely he is to explore these feelings deeply with the helper, (3) the more likely he is to listen fully to and act upon any advice offered him by the helper. (A. P. Goldstein, 1971, p. 19)

This is a widely held view. Much of the clout in diverse psychotherapies lies in the relationship, whether the therapy is psychoanalytic (Ferreira & Rosen, 1982), client-centered (Rice, 1982), cognitive (Arnkoff, 1982), behavioral (R. J. Morris & Magrath, 1982), gestalt (Greenberg, 1982), family (Stahmann & Harper, 1982), or group (Fuhriman & Barlow, 1982). What is the patient–therapist relationship? Several key ingredients are frequently mentioned, one cluster of which could be described as attraction, liking, and warmth, experienced reciprocally patient to therapist, therapist to patient. In addition to these emotional components, a good therapist–patient relationship also captures and responds to the role of the helper as a competent expert. Thus, one often finds mentioned such key relationship ingredients as patient perception of the therapist as trustworthy, credible, and skilled in the task. It appears, therefore, that a good therapist–patient relationship can best be defined as a relationship fully characterized by attitudes and behaviors reflecting both warmth and respect (A. P. Goldstein, 1971; Lambert, 1982).

In their continuing effort to better understand how psychotherapy works, and to make it work better, applied psychologists have conducted many investigations of the therapist–patient relationship, focusing in particular on its association with patient improvement. Does a good therapist–patient relationship actually lead to a better outcome for the patient, as so many psychotherapists claim? The answer appears to be a confident yes. The scientific study of psychotherapy began in 1950, and, throughout its approximately 35-year history, there have been many research demonstrations of the relationship–outcome association (Bown, 1954; Goldstein, 1971; J. Hunt, Ewing, LaForge, & Gilbert, 1959; Lambert, 1982; Parloff, 1961; Vander Veen, 1965).

The potency of the therapeutic relationship for patient change can be demonstrated not alone by research such as that just cited showing that a good relationship leads to positive patient change. The relationship–outcome connection can be further demonstrated by showing that a poor relationship results not in patient improvement but in deterioration. Lambert (1982) disclosed that:

> . . . the recognition that some patients are actually made worse by the therapy that was intended to help them has proved even more informative about the active ingredients in psychotherapy. Empirical research has helped us to identify several specific ways in which people are made worse by or hurt in therapy. Reviews of this phenomenon . . . suggest that, although some negative out-